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/ **Practice Exam - 1**

Correct Answer

Next Question

Practice Exam - 1

Question 27 of 117



A 60-year-old man with ischemic cardiomyopathy and left ventricular ejection fraction of 20-25%, VT with a subcutaneous ICD, paroxysmal atrial fibrillation/flutter that was ablated, hypertension, hyperlipidemia, CKD stage 3b, presents to emergency room with fatigue, sweating, and dyspnea. Patient reports adherence to recommended fluid and salt restrictions and medication regimen which includes:

apixaban 5 mg twice daily, amiodarone 200 mg daily, carvedilol 12.5mg twice daily, lisinopril 20mg daily, and spironolactone 25mg daily.

On examination his temperature is 100.1F, blood pressure is 117/55 mm Hg, heart rate is 114 bpm, respiratory rate is 14/min, and oxygen saturation rate is 98% while breathing ambient air. BMI is 38kg/m2. He is diaphoretic, anxious appearing; there is no appreciable JVD. His cardiovascular, pulmonary and abdominal examinations are unremarkable. Distal extremities are warm to touch.

Laboratory studies reveal sodium of 138 mEq/L, potassium of 4.0 mEq/L, glucose 106 mg/dL, creatinine of 1.63 mg/dL white blood cell count of 10.38 K/uL, and hemoglobin of 10.0 g/dL, ferritin 300 µg/L. Transthoracic echocardiogram shows a stable left ventricular ejection fraction of 20-25%.

However right ventricular dilation of 5.3cm with reduced function on visual assessment is now present. TAPSE is measured at 0.8cm, lateral s' is 0.08m/sec. IVC is normal in size with blunted respirophasic diameter changes. Right heart catheterization shows RA 12/12/8, RV 74/7/15, PA 76/32/47, PCWP 17/16/14, Fick CO 10.3L/min, CI 5L/min/m2, without shunt present, SVR 640 dyn·sec·cm⁻⁵, PVR 3.2 Wood's units.

Which of the following is the best next step in the management of this patient?

A. Obtain transferrin saturation

 B. Refer for infectious disease evaluation

C. Refer patient to bariatric clinic

 D. Obtain Thyroid Function Tests

Testing point: Recognize amiodarone toxicity with thyrotoxicosis, high output heart failure and pulmonary hypertension.

Answer: D. Obtain thyroid function tests.

This patient has high output heart failure. The stem gives clues here to the most likely cause, with a number of distractors. In the setting of long-standing ventricular arrhythmia and chronic use of amiodarone, thyrotoxicosis secondary to amiodarone is very high on the differential, dangerous, and should immediately be ruled out.

Answer A: Obtain transferrin saturation. The patient's hemoglobin is not so low that suspicion for high output HF secondary to anemia is a primary concern. The ferritin level is less suggestive of iron-deficiency anemia, although can be misleadingly elevated in heart failure, especially acute decompensated heart failure, as it is an acute phase reactant. Obtaining transferrin is reasonable in the course of care, but is not the most likely cause of high output failure in this patient.

Answer B: Refer for infectious disease evaluation The elevated CO/CI and low SVR could be consistent with sepsis, however the patient's presentation is not suggestive of severe systemic distributive shock. He has a normal blood pressure and low risk for serious infection.

Answer C: Refer patient to bariatric clinic Severe obesity and the increase in vascular cross sectional area and circulatory demand can on rare occasions cause high output failure with CO/CI and low SVR. However, in the setting of chronic amiodarone use, this is not the most likely possibility, and the possibility of thyrotoxicosis needs immediate attention.

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